

DYNIDAZOL

	DESCRIPTION	CONTENT
Dynidazol Tablet 200mg MAL19987009AZ	TABLET Colour : White Shape : Round & Biconvex	Each tablet contains: Metronidazole 200 mg
Dynidazol Tablet 400mg MAL19973017AZ	TABLET Colour : White Shape : Round, Flat & Scored	Each tablet contains: Metronidazole 400 mg

PHARMACODYNAMICS:

Metronidazole is a 5-nitroimidazole derivative with activity against anaerobic protozoa and anaerobic bacteria; it also has a radiosensitising effect on hypoxic tumour cells. Its mechanism of action is thought to involve interference with DNA by a metabolite in which the nitro group of Metronidazole has been reduced.

PHARMACOKINETICS:

Metronidazole is readily absorbed following administration by mouth and bioavailability approaches 100%. Peak plasma concentrations of approximately 5 and 10 µg per mL are achieved an average of one hour after single doses of 250 and 500 mg respectively. Absorption may be delayed, but is not reduced overall by administration with food. Metronidazole is widely distributed. It appears in most body tissues and fluids including biles, bone, breast milk, cerebral abscesses, cerebrospinal fluid, liver and liver abscesses, saliva, seminal fluid, and vaginal secretions, and achieves concentrations similar to those in plasma. It also crosses the placenta and rapidly enters the foetal circulation. No more than 20% is bound to plasma proteins. Metronidazole is metabolised in the liver by side-chain oxidation and glucuronide formation. The plasma elimination half-life of Metronidazole is about 8 hours; that of the hydroxy metabolite is slightly longer. The half-life of Metronidazole is reported to be longer in neonates and in patients with severe liver disease; that of the hydroxy metabolite is prolonged in patients with renal failure. The majority of a dose of Metronidazole is excreted in the urine, mainly as metabolites; a small amount appears in the faeces. Depending on the assay method used, up to 80% of a dose has been recovered in the urine within 48 hours.

INDICATION:

Metronidazole is used in the treatment of susceptible protozoal infections and in the treatment and prophylaxis of anaerobic bacterial infections.

Metronidazole is active against several protozoa including *Balantidium coli*, *Blastocystis hominis*, *Entamoeba histolytica*, *Giardia intestinalis* (*Giardia lamblia*), and *Trichomonas vaginalis*. Most obligate anaerobic bacteria, including *Bacteroides* and *Clostridium* spp., are sensitive in vitro to Metronidazole. It is bactericidal.

Specific bacterial infections treated with Metronidazole include bacterial vaginosis (nonspecific vaginitis), acute necrotising ulcerative gingivitis (Vincent's infection).

RECOMMENDED DOSE:

All to be taken with or after food.

Amoebiasis, Balantidiasis, Blastocystis hominis infection: Adults, 400 to 800 mg three times daily by mouth for 5 to 10 days. Children aged 1 to 3 years may be given one-quarter, those aged 3 to 7 years one-third, and those aged 7 to 10 years one-half the adult dose; alternatively 35 to 50 mg per kg body-weight daily in divided doses has been suggested.

Giardiasis: Adults: The usual dose of Metronidazole is 2 g daily by mouth as a single dose for 3 successive days. Dosage for children is proportional, as for amoebiasis (above).

Vaginal Trichomoniasis, Bacterial Vaginosis: A single 2 g dose or as a 7-day course of 200 mg three times daily or 400 mg twice daily. Children 1 to 3 years: 50 mg three times daily. 3 to 7 years: 100 mg twice daily and 7 to 10 years: 100 mg three times daily.

Acute Necrotising Ulcerative Gingivitis, Acute Dental Infections: 200 mg three times daily for three days.

Anaerobic Bacterial Infections: Initial dose of 800 mg, followed by 400 mg every 8 hours, usually for about 7 days.

For Prevention Of Postoperative Anaerobic Bacterial Infections: 400 mg every 8 hours for 3 to 4 days followed postoperatively by intravenous or rectal administration until oral therapy is possible; shorter pre-operative course and oral doses of up to 1 g have also been used.

ROUTE OF ADMINISTRATION:

FOR ORAL USE ONLY

CONTRAINDICATION:

Known hypersensitivity to Metronidazole.

WARNING AND PRECAUTIONS:

Metronidazole should be used with great care in patients with blood dyscrasias or with active disease of the central nervous system. All patients receiving Metronidazole for more than 10 days should be monitored and treatment discontinued if signs of peripheral neuropathy or CNS toxicity develop.

Patients with severely impaired hepatic function metabolise Metronidazole slowly. Close monitoring for toxicity, as well as reduction in dose, as suggested by the physician may be required.

Cases of severe hepatotoxicity/acute hepatic failure, including cases with a fatal outcome with very rapid onset after treatment initiation in patients with Cockayne syndrome have been reported with products containing Metronidazole for systemic use. In this population, Metronidazole should therefore be used after careful benefit-risk assessment and only if no alternative treatment is available. Liver function test must be performed just prior to the start of therapy, throughout and after end of treatment until liver function is within normal ranges, or until the baseline values are reached. If the liver function tests become markedly elevated during treatment, the drug should be discontinued.

Patients with Cockayne syndrome should be advised to immediately report any symptoms of potential liver injury to their physician and stop taking Metronidazole.

DRUG INTERACTIONS:

When given in conjunction with alcohol, Metronidazole may provoke a disulfiram-like reaction in some individuals; reactions have occurred after the administration of pharmaceutical preparations formulated with alcohol, including injections, as well as after drinking alcohol. Acute psychoses or confusion have been associated with the concomitant use of Metronidazole and Disulfiram.

Some potentiation of anticoagulants therapy has been reported when Metronidazole has been used with the warfarin-type oral anticoagulants. Dosage of the latter may require reducing. Prothrombin times should be monitored. There is no interaction with heparin.

Lithium retention accompanied by evidence of possible renal damage has been reported in patients treated simultaneously with Lithium and Metronidazole. Lithium treatment should, therefore, be tapered or withdrawn before administering Metronidazole. Plasma concentration of Lithium, creatinine and electrolytes should be monitored in patients under treatment with Lithium while they receive Metronidazole.

Patients receiving phenobarbitone metabolise Metronidazole at a much greater rate than normal reducing the half-life to approximately 3 hours.

There is some evidence that Phenytoin might accelerate the metabolism of Metronidazole.

Cimetidine has increased plasma concentration of Metronidazole and might increase the risk of neurological side-effects.

PREGNANCY AND LACTATION:

It is suggested that the use of Metronidazole should be avoided during pregnancy, especially the first trimester. Some authorities consider that women taking Metronidazole should not breast-feed their babies.

SIDE EFFECTS:

Serious adverse reactions occur very rarely with standard recommended regimens. Unpleasant taste in the mouth, flurried tongue, nausea, vomiting, gastrointestinal disturbance and urticaria and angioedema occur occasionally. Anaphylaxis may occur rarely. Drowsiness, dizziness, headache, ataxia, skin rashes, pruritus, darkening of the urine (due to Metronidazole metabolite) have been reported but very rarely. During intensive and/or prolonged Metronidazole therapy, a few instances of peripheral neuropathy or transient epileptiform seizures have been reported. In most cases, neuropathy disappeared after treatment was stopped or when dosage was reduced. A moderate leucopenia has been reported in some patients but the white cell count has always returned to normal before or after treatment has been completed.

SYMPTOMS AND TREATMENT OF OVERDOSE:

Overdose may cause dizziness, headache, gastrointestinal disturbance e.g. nausea, vomiting, diarrhoea, stomach pain or cramps. There is no specific antidote; treatment for Metronidazole overdose should be symptomatic and supportive.

PACKING/PACK SIZE(S):

Blister pack of 100x10's.

**JAUHI UBAT DARIPADA KANAK-KANAK
KEEP OUT OF REACH OF CHILDREN**

MANUFACTURER/PRODUCT REGISTRATION HOLDER:

DYNAPHARM (M) SDN. BHD. (65683-V)

2497, Mk. 1, Lorong Perusahaan Baru 5,
Kawasan Perusahaan Perai 3,
13600 Perai, Pulau Pinang, MALAYSIA

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